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First Aid

Kenya Medical Training College

By

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COURSE OUTLINE

Module Learning Outcomes

By the end of this module, the learner shall be able to:

1. Describe the tasks of a first aider in an emergency situation
2. Evaluate a casualty in order to get appropriate first aid treatment
3. Carry out appropriate first aid treatment necessary to improve casualty's condition
4. Provide necessary first aid intervention in emergency conditions
5. Institute appropriate treatment measures for various body injuries
6. Perform correctly all skills pertaining to specific casualties

Module Content

Overview of First Aid - Introduction, Principles and practices, Aims of first aid, responsibilities of a first aider, scope, eight qualities of a first aider, features and contents of the first aid kit. Incident management: AMEGA principles (assessing the area, managing the incident, emergency aid, get help, deal with aftermath). **Casualty management -** The emergency process (primary survey), DRABC, Secondary Survey/ Top to Toe. M**anagement of conditions of body systems -** conditions of the respiratory, circulatory and central nervous systems. M**anagement of various emergency conditions** - Medical emergencies conditions: Diabetes, hypoglycemia, hyperglycemia, allergy, fever, meningitis, headache, ear ache and tooth ache, abdomen pain, vomiting and diarrhoea, poisons, bites and stings, foreign bodies, Emergency child birth: Stages of labour, signs and symptoms of labour. **Management of body injuries:** fractures, dislocations, wounds, sprains, strains, head injuries, crush injuries, burns and scalds, extremes of temperatures. **Skills demonstration and practice:** External chest compression and cardio pulmonary resuscitation (CPR), Artificial Ventilations, Recovery position, incidents management and casualty attention, bandaging and dressings, lifting, moving, carrying and transportation of a casualty, Ambulance drill, General casualty management

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First Aid

First aid is the assistance given to any person suffering a sudden illness or injury,[1] with care provided to preserve life, prevent the condition from worsening, and/or promote recovery. It includes initial intervention in a serious condition prior to professional medical help being available, such as performing CPR while awaiting an ambulance, as well as the complete treatment of minor conditions, such as applying a plaster to a cut. First aid is generally performed by the layperson, with many people trained in providing basic levels of first aid, and others willing to do so from acquired knowledge. Mental health first aid is an extension of the concept of first aid to cover mental health.

There are many situations which may require first aid, and many countries have legislation, regulation, or guidance which specifies a minimum level of first aid provision in certain circumstances. This can include specific training or equipment to be available in the workplace (such as an Automated External Defibrillator), the provision of specialist first aid cover at public gatherings, or mandatory first aid training within schools. First aid, however, does not necessarily require any particular equipment or prior knowledge, and can involve improvisation with materials available at the time, often by untrained persons.[2]

Aims

The key aims of first aid can be summarised in three key points, sometimes known as 'the three P's'

- **Preserve life**: The overriding aim of all medical care which includes first aid, is to save lives and minimize the threat of death.
- **Prevent further harm**: Prevent further harm also sometimes called **prevent the condition from worsening**, or **danger of further injury**, this covers both external factors, such as moving a patient away from any cause of harm, and applying first aid techniques to prevent worsening of the condition, such as applying pressure to stop a bleed becoming dangerous.
- **Promote recovery**: First aid also involves trying to start the recovery process from the illness or injury, and in some cases might involve completing a treatment, such as in the case of applying a plaster to a small wound.

Certain skills are considered essential to the provision of first aid and are taught ubiquitously. Particularly the "ABC"s of first aid, which focus on critical life-saving intervention, must be

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rendered before treatment of less serious injuries. ABC stands for Airway, Breathing, and Circulation. The same mnemonic is used by all emergency health professionals. Attention must first be brought to the airway to ensure it is clear. Obstruction (choking) is a life-threatening emergency. Following evaluation of the airway, a first aid attendant would determine adequacy of breathing and provide rescue breathing if necessary. Assessment of circulation is now not usually carried out for patients who are not breathing, with first aiders now trained to go straight to chest compressions (and thus providing artificial circulation) but pulse checks may be done on less serious patients.

Some organizations add a fourth step of "D" for Deadly bleeding or Defibrillation, while others consider this as part of the Circulation step. Variations on techniques to evaluate and maintain the ABCs depend on the skill level of the first aider. Once the ABCs are secured, first aiders can begin additional treatments, as required. Some organizations teach the same order of priority using the "3Bs": Breathing, Bleeding, and Bones (or "4Bs": Breathing, Bleeding, Burns, and Bones). While the ABCs and 3Bs are taught to be performed sequentially, certain conditions may require the consideration of two steps simultaneously. This includes the provision of both artificial respiration and chest compressions to someone who is not breathing and has no pulse, and the consideration of cervical spine injuries when ensuring an open airway.

Preserving life

In order to stay alive, all persons need to have an open airway—a clear passage where air can move in through the mouth or nose through the pharynx and down into the lungs, without obstruction. Conscious people will maintain their own airway automatically, but those who are unconscious (with a GCS of less than 8) may be unable to maintain a patent airway, as the part of the brain which automatically controls breathing in normal situations may not be functioning.

If the patient was breathing, a first aider would normally then place them in the recovery position, with the patient leant over on their side, which also has the effect of clearing the tongue from the pharynx. It also avoids a common cause of death in unconscious patients, which is choking on regurgitated stomach contents.

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The airway can also become blocked through a foreign object becoming lodged in the pharynx or larynx, commonly called choking. The first aider will be taught to deal with this through a combination of ‘back slaps’ and ‘abdominal thrusts’.

Once the airway has been opened, the first aider would assess to see if the patient is breathing. If there is no breathing, or the patient is not breathing normally, such as agonal breathing, the first aider would undertake what is probably the most recognized first aid procedure—cardiopulmonary resuscitation or CPR, which involves breathing for the patient, and manually massaging the heart to promote blood flow around the body.

Promoting recovery

The first aider is also likely to be trained in dealing with injuries such as cuts, grazes or bone fracture. They may be able to deal with the situation in its entirety (a small adhesive bandage on a paper cut), or may be required to maintain the condition of something like a broken bone, until the next stage of definitive care (usually an ambulance) arrives.

The role of a first aider

First aid is the help given to someone who is injured or ill, to keep them safe until they can get more advanced medical treatment by seeing a doctor, health professional or go to hospital.

The role of a first aider is to give someone this help, while making sure that they and anyone else involved are safe and that they don’t make the situation worse.

If you think someone needs your help, these are the seven things you need to do as a first aider:

1. Assess the situation quickly and calmly:

- Safety: Are you or they in any danger? Is it safe for you to go up to them?
- Scene: What caused the accident or situation? How many casualties are there?
- Situation: What’s happened? How many people are involved and how old are they? What do you think the main injuries could be?

2. Protect yourself and them from any danger:

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- Always protect yourself first - never put yourself at risk
- Only move them to safety if leaving them would cause them more harm
- If you can't make an area safe, call 999/112 for emergency help

3. Prevent infection between you and them:

- Wash your hands or use alcohol gel
- Wear disposable gloves
- Don't touch an open wound without gloves on
- Don't breathe, cough or sneeze over a wound or a casualty

4. Comfort and reassure:

- Stay calm and take charge of the situation
- Introduce yourself to them to help gain their trust
- Explain what's happening and why
- Say what you're going to do before you do it

5. Assess the casualty:

- If there's more than one casualty, help those with life-threatening conditions first
- Start with the Primary Survey and deal with any life-threatening conditions
- Then, if you've dealt with these successfully, move on to the Secondary Survey

6. Give first aid treatment:

- Prioritise the most life-threatening conditions
- Then move on to less serious ones
- Get help from others if possible

7. Arrange for the right kind of help:

- Call for an ambulance if you think it's serious
- Take or send them to hospital if it's a serious condition but is unlikely to get worse
- For a less serious condition call 111 for medical advice (in England)
- Suggest they see their doctor if they're concerned about a less serious condition
- Advise them to go home to rest, but to seek help if they feel worse
- Stay with them until you can leave them in the right car

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How to assess a casualty

If you think someone needs your help, before you do anything you need to check the situation is safe and assess what's wrong with them. These checks are broken into two stages – called The Primary and Secondary Survey:

The Primary Survey

First, you need to look and see if they have any injuries or conditions that could be immediately life-threatening, and deal with these as quickly as possible. To do this, the letters DR. ABC will help you to remember to check for Danger, Response, Airway, Breathing and Circulation.

The Secondary Survey

If you manage to deal with anything life-threatening successfully, or there is nothing life-threatening to deal with, then you can assess someone in more detail for any other injuries or conditions.

The Primary Survey

The Primary Survey is a quick way for you to find out if someone has any injuries or conditions which are life-threatening. If you follow each step methodically, you can identify each life-threatening condition and deal with it in order of priority.

Use the letters **DR. ABC** to remember the steps: **D**anger, **R**esponse, **A**irway, **B**reathing and **C**irculation.

You'll need to go through the Primary Survey every time you help someone, and make sure you don't get distracted by anything else. Only move onto the Secondary Survey, if you've already done the Primary Survey and succeeded in dealing with any life-threatening conditions.

Danger:

If someone needs help, before you go up to them check – is it safe?

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- **No:** If you can see or hear any danger nearby, for you or them, like broken glass or oncoming traffic, then make the situation safe before you get any closer
- **Yes:** If you can't see or hear any danger then it is safe to go up to them.

Response:

Do they respond when you ask them: 'Are you alright?' or if you say: 'Open your eyes!'

- **No:** If they don't respond, pinch their ear lobe or gently shake their shoulders, or with a child - tap their shoulder, and with a baby - tap their foot. If they still don't respond, then you can presume they're unresponsive and move on to the next stage – **Airway**. Someone who's unresponsive should always take priority so you should treat them first and as quickly as possible.
- **Yes:** If they respond by making eye contact with you or some gesture then you know that they're responsive and you can move on to the next stage – **Airway**.

Airway:

Is their airway open and clear?

No:

o Responsive: If they're responsive, treat them for conditions that may be blocking their airway, such as choking. Only move on to the next stage – **Breathing** – once their airway is open and clear.

o Unresponsive: If they're unresponsive, tilt their head and lift their chin to open their airway. Only move on to the next stage – **Breathing** – once their airway is open and clear.

• **Yes:** If their airway is open and clear, move on to the next stage – **Breathing.**

Breathing:

Are they breathing normally? You need to look, listen and feel to check they're breathing.

• **No:**

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o Responsive: If they're conscious, treat them for whatever is stopping them breathing, for example, an obstructed airway. Then go to the next stage – **Circulation**

o Unresponsive: If they're unresponsive and not breathing, call 999/112 for an ambulance, or get someone else to call if possible, and start giving chest compressions and rescue breaths CPR – cardiopulmonary resuscitation. If this happens you probably won't move on to the next stage as the casualty needs resuscitation.

• **Yes:** If they are breathing normally, move on to the next stage – circulation.

Circulation:

Are there any signs of severe bleeding?

• **Yes:** If they're bleeding severely, control the bleeding with your gloved fingers, dressing or clothing, call 999/112 for an ambulance and treat them to reduce the risk of them going into shock.

• **No:** If they aren't bleeding, and you're sure you have dealt with any life-threatening conditions, then you can move on to the Secondary Survey, to check for any other injuries or illnesses.

The Secondary Survey

Only move onto the Secondary Survey if you've already done the Primary Survey and succeeded in dealing with any life-threatening conditions.

Then you can start questioning the casualty about what's happened and carefully check someone for any other injuries or illnesses. If you can, jot down everything you find out and give all this information to the emergency services or whoever takes responsibility for the child, like a parent.

You need to find out:

• **History:** Question them about what happened leading up to them injuring themselves or feeling unwell? Ask those around them too and write everything down if they can.

• **Symptoms:** What symptoms do they tell you they have?

• **Signs:** Check them over from head to toe. What signs do you find on their body?

History

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Event history

Ask them to describe exactly what happened leading up to them feeling unwell or injuring themselves.

You can ask other people near the scene too and also look for clues. For example, if they've had a car accident the impact on the car will help you work out what type of injury they could have.

Medical history

Then, ask them to tell you their medical history. Use the word **AMPLE** to remember all the things you need to ask them:

Allergy – do they have any allergies?

Medication – are they taking any regular or prescribed medication?

Previous medical history – did they already have any conditions?

Last meal – when did they last eat something?

Event history – what happened?

Symptoms

Ask them to give you as much detail as possible about how they feel. Listen carefully to what they say and make notes, if possible.

Here are they key questions to ask them:

•Can they feel any pain?

•Can they describe the pain, e.g. is it constant or irregular, sharp or dull?

•What makes the pain better or worse?

•When did the pain start?

Signs

Check the casualty over from head to toe, using all your senses – look, listen, feel and smell.

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You may have to loosen, open, cut away or remove clothing. Ask their permission to do this and make sure you're sensitive and discreet.

Make a note of any minor injuries as you go. Only return to these when you have finished checking the whole body, to make sure you don't miss any more serious injuries.

Head to toe examination

Breathing and pulse: How fast and strong is their breathing and pulse?

Bleeding: Check the body from head-to-toe for any bleeding.

Head and neck: Is there any bleeding, swelling, sensitivity or a dent in the bone, which could mean a fracture?

Ear: Do they respond when you talk to them? Is there any blood or clear fluid coming from either ear? If so, this could mean a serious head injury.

Eyes: Are they open? What size are their pupils (the black bit)? If they're different sizes this could mean a head injury.

Nose: Is there any blood or clear fluid coming from the nostrils? This could mean a serious head injury.

Mouth: Check their mouth for anything which could block their airway. Look for mouth injuries or burns in their mouth and anything unusual in the line of their teeth.

Skin: Note the colour and temperature of their skin. Pale, cold, clammy skin suggests shock. A flushed, hot face suggests fever or heatstroke. A blue tinge suggests lack of oxygen from an obstructed airway, poor circulation, or asthma.

Neck: Loosen any clothing around their neck to look for signs like a medical warning medallion or a hole in their windpipe. Run your fingers down their spine without moving it to check for any swelling, sensitivity or deformity.

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Chest: Check if the chest rises easily and evenly on each side as they breathe. Feel the ribcage to check for any deformity or sensitivity. Note if breathing is difficult for them or painful in any way.

Collar bone, arms and fingers: Feel all the way along the collar bones to the fingers for any swelling, sensitivity or deformity. Check they can move their elbows, wrists and fingers.

Arms and fingers: Check they don't have any unusual feeling in their arms or fingers. If their fingertips are pale or greyish-blue this could suggest their blood isn't circulating properly. Also look for any needle marks on the forearms, which suggest drug use. See if they have a medical warning bracelet.

Spine: If they've lost any movement or sensation in their legs or arms. Don't move them to check their spine as they may have a spinal injury. Otherwise, gently put your hand under their back and check for any swelling or soreness

Abdomen: Gently feel their abdomen to check for any signs of internal bleeding, like stiffness or soreness, on each side.

Hips and pelvis: Feel both hips and the pelvis for signs of a fracture. Check their clothing for any signs of incontinence, which may suggest a spinal injury or bladder injury, or bleeding from body openings, which may suggest a pelvic fracture.

Legs: Check the legs for any bleeding, swelling, deformity or soreness. Ask them to raise one leg and then the other, and to move their ankles and knees.

Toes: Check their movement and feeling in their toes. Compare both feet and note the colour of the skin: greyish-blue skin could suggest a problem with their circulation or an injury due to cold, like hypothermia.

Difficult first aid situations

When it comes to first aid, assessing the situation properly is crucial. Call for help early on.

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Incidents involving traffic, fire, electricity or water can put your own life at risk. Be sure it is safe to approach.

Important questions to ask yourself

Safety

• What are the dangers and are you or the casualty still in any danger?

• Have you got any protective clothing or equipment on?

• Is it safe for you to go up to them?

Scene

• What has caused the accident or situation?

• How many casualties are there?

Situation

• What happened?

• How many people are involved and how old are they?

• What do you think the main injuries could be?

Traffic incidents

How to make the area safe

- Call for help – 999 or 112.
- Park safely.
- Set up warning triangles 45m away in each direction.
- Make vehicles safe by turning off the ignition.
- Stop upright vehicles from moving by applying a handbrake, putting in gear or putting blocks in front of the wheels.
- Look out for physical dangers like oncoming traffic.
- Tell the emergency services if any power lines have been damaged, fuel has been spilt or if any vehicles have hazard signs showing they contain harmful substances.

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How to assess the casualties

- Assume that anyone who's been in a road traffic accident could have a neck or spinal injury.
- If possible, treat them in the position you find them in, supporting their head and neck all the time and wait for the emergency services to come.
- Search the area to make sure you don't miss anyone who have may been thrown or who has wandered off.
- Ask bystanders to help.
- If someone's trapped inside or under a vehicle, you'll need to wait for the fire service to get them out.

Fires

How to leave a burning building

- If you see or suspect a fire in a building, set off the first fire alarm you see.
- Call 999 or 112.
- Try to help others out of the building but don't put yourself at risk.
- Close doors behind you to help stop the fire from spreading.
- Use the fire exits and look for assembly points outside.
- Don't use lifts.

What to do if someone's clothing is on fire

If someone's clothing is on fire, remember – **Stop**, **Drop** and **Roll**.

- **Stop** them from panicking or running or going outside because any movement or breeze will fan the flames.
- **Drop** them to the ground and wrap them tightly in a fire blanket or heavy material like a coat, curtain, woollen blanket or rug.
- **Roll** them along the ground to smother the flames until they go out – then treat any burns by cooling them as soon as possible.
- Call for emergency help.

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Electrical incidents

Common injuries from electrical incidents

Cardiac arrest – if someone experiences an electric current through their body it may stun them and stop their breathing and heartbeat.

Burns – an electric current can give someone burns where it enters and exits the body.

Fractures and spinal injuries – a direct current (DC) can cause a large muscular contraction that often throws someone and can result in injuries like fractures and spinal injuries.

Electrically charged – an alternating current (AC) causes muscular spasms which can lock their grasp so that they can't let go. This means they are electrically charged so you mustn't touch them or you'll risk electrocution too. Instead you need to break their contact with the source of electricity.

How to break contact with electricity

Don't touch the person. Call for help.

If you can, stop the source of electricity by:

- turning it off
- pulling out the plug, or
- switching off the current at the mains.

If you can't stop the source of electricity, move the casualty away from it **without touching them directly**. Don't use anything metal as this will conduct the electricity.

- • Stand on material like a wooden box, a plastic mat or a book.
- • Use something wooden like a pole, broom or stool – **nothing metal** – to push their limbs away from the source.
- • Or loop some rope around their ankles or under their arms – **making absolutely sure you don't touch them** – and pull them away from the source.

Once you're sure the casualty is clear of the electric current, carry out the primary survey, treat their injuries in order of priority and call 999 or 112 for medical help.

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Water incidents

Common conditions caused by water incidents include

Cardiac arrest – if someone's suddenly falls into cold water it can make their heart stop, known as cardiac arrest.

Hypothermia – being in cold water can cause hypothermia.

Shock – being in cold water can make the conditions of shock worse.

Drowning – if someone inhales water this can block the airway causing them to drown.

Heart problems – the effort of swimming can also put a strain on the heart.

How to rescue someone from water

Call for help

Getting the casualty out of the water

- Stay on dry land yourself and throw them a life belt if one is available, or hold out a stick, a branch or rope for them to grab and then pull them out of the water.
- If they can't pull themselves out because they're unresponsive, and it's safe for you to go in the water, wade or swim to the casualty and tow them to the bank, keeping them as upright as you can.
- If you can't get the casualty out of the water safely, wait for the ambulance.

When the casualty is out of the water

- If they're unresponsive, open their airway and check for breathing.
- If they are not breathing, begin resuscitation (CPR).
- If they're responsive, try to shield them from the wind and treat them for drowning and then for hypothermia.
- Take or send them to hospital even if they seem to be better. If you are worried about how serious the casualty's condition is then call 999 or 112 for medical help, if you haven't previousy done so.

Breathing problems

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This section of first aid advice from St John Ambulance covers the most common situations relating to breathing problems. Watch our professional videos, see our easy to follow illustrations and read our expert first aid advice.

We help you to know what to do if someone is choking, whether they are a baby, a child or an adult. Get advice on how to treat someone who is suffering from an asthma attack. We provide information on what to do if a baby or child has croup, give advice for when someone is hyperventilating and how to treat someone for the effects of drowning.

Breathing problems

- Asthma attack
- Choking – adults
- Choking advice for parents (child and baby)
- Croup
- Drowning
- Hyperventilation

Asthma attack

In an asthma attack, the muscles of the air passages in the lungs go into spasm. This makes the airways narrower, making it difficult to breathe.

Sometimes something specific can trigger an attack, such as an allergy, a cold, or cigarette smoke. At other times, someone may have a sudden attack with no obvious trigger.

People with asthma usually deal with their own attacks by using a blue reliever inhaler at the first sign of an attack. But if someone doesn't have an inhaler, or the attack is severe, you may need to help.

What to look for - Asthma attacks

If you think someone is having an asthma attack, these are the five key things to look for:

1. Difficulty breathing or speaking
2. Wheezing
3. Coughing
4. Distress
5. Grey-blue tinge to the lips, earlobes and nailbeds (known as cyanosis).

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What you need to do - Asthma attacks

- First, reassure them and ask them to breathe slowly and deeply which will help them control their breathing.
- Then help them use their reliever inhaler straight away. This should relieve the attack.
- Next, sit them down in a comfortable position.
- If it doesn't get better within a few minutes, it may be a severe attack. Get them to take one or two puffs of their inhaler every two minutes, until they've had 10 puffs.
- If the attack is severe and they are getting worse or becoming exhausted, or if this is their first attack, then call 999/112 for an ambulance.
- Help them to keep using their inhaler if they need to. Keep checking their breathing, pulse and level of response.
- If they lose responsiveness at any point, open their airway, check their breathing and prepare to treat someone who's become unresponsive

Choking adults

Choking is when your airway gets blocked and you can't breathe properly.

When someone chokes, the airway can either be partly or fully blocked. If it's a mild blockage, they should be able to clear it themselves by coughing. If it's a severe blockage, they won't be able to cough so without anyone's help they'll become unresponsive.

But if they do become unresponsive, their throat muscles could relax and open the airway enough for you to give rescue breaths – be prepared to give rescue breaths and chest compressions.

Choking adult – what to look for

If you think someone is choking, ask them: 'Are you choking?' to check they're not suffering from something else. Can they speak, cry, cough or breathe?

If they can, they should be able to clear their throat on their own by coughing, so encourage them to cough.

If they can't cough or make any noise, it's serious.

Choking adult – what you need to do

Help clear their throat with these three steps.

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Step 1 of 4: Cough it out

- Encourage them to cough. If this doesn't clear the obstruction, support their upper body with one hand and help them lean forward.

Step 2 of 4: Slap it out

- If coughing doesn’t work, help the casualty bend forward.
- Use the heel of your hand to give up to five sharp back blows between their shoulder blades.
- Check their mouth to see if there’s anything in there and, if there is, get them to pick it out.

Step 3 of 4: Squeeze it out

- • If back blows don’t work, give up to five abdominal thrusts. Stand behind them.
- • Link your hands between their tummy button and the bottom of their chest, with your lower hand clenched in a fist.
- • Pull sharply inwards and upwards.

Step 4 of 4: Call for help

- If they’re still choking, call for medical help.
- Once you’ve called, continue steps 2 and 3 – back blows and abdominal thrusts – until what’s in there has cleared, help arrives or they become unresponsive.
- If they become unresponsive at any stage, open their airway and check their breathing.

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- If they're not breathing, start chest compressions and rescue breaths (CPR - cardiopulmonary resuscitation) to try to release whatever's stuck in there. Follow the instructions for treating someone who's unresponsive and not breathing.

Croup in children

Croup is a childhood condition that makes it difficult to breathe. Croup is caused by the inflammation of the windpipe (trachea) and the voice box (larynx).

Children with croup have difficulty breathing, a distinctive barking cough and may have a croaky voice.

Croup can usually be diagnosed by a GP and treated at home. If an attack of croup is severe and doesn't go away, call 999 or 112 for emergency help.

There is a small risk that they may have a rare but similar condition called epiglottitis. The epiglottis is a small flap of tissue at the base of the tongue which keeps food from going down the windpipe when swallowing. If this gets infected and swollen it can block the airway.

If you think a child might have epiglottitis, it's a medical emergency and you need to call 999 or 112 immediately.

What to look for - croup

If you think a child may have croup, these are the four key things to look for:

1. Distressed breathing
2. A short barking cough
3. Rasping noise and croaky voice
4. Blue-grey skin

What you need to do - croup

- Stay calm and don't panic, as this will alarm them and probably make the attack of croup worse.
- Sit them on your knee, supporting their back, and reassure them calmly.
- If the croup is severe, call 999 or 112 for emergency medical help.
- If they have a high temperature too, then suspect epiglottitis and call 999 or 112 immediately.
- When you're waiting for help to arrive, keep checking the child's breathing, pulse and level of response.

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Drowning

Drowning is when someone is unable to breathe because their nose and mouth are submerged in water, or in another liquid.

When someone's drowning, it may not look like the violent, splashing call for help that most people expect from watching TV. When someone’s actually drowning, they won’t able to make any noise, so can easily go unnoticed, even if friends or family are nearby.

What to look for - Drowning

If someone has been rescued from drowning you need to check if they’re breathing or not.

If they aren't breathing, then you’ll need to give **CPR (cardio pulmonary resuscitation)** straight away.

What you need to do - Drowning

- As soon as the casualty has been rescued from the water, check if they’re breathing.
- Ask someone to call 999 or 112 for medical help.
- If the person is unresponsive and not breathing, give them five initial rescue breaths before starting CPR.
- Once you’ve done this, start CPR: 30 chest compressions, then two rescue breaths. Keep giving CPR until help arrives, the casualty regains responsiveness, or you’re too exhausted to keep going.
- If they start breathing again at any time, treat them for hypothermia by covering them with warm clothes and blankets. If they recover completely, replace their wet clothes with dry ones.
- Keep checking breathing, pulse and level of response until help arrives.

Hyperventilation

Hyperventilation is excessive breathing, normally caused by extreme anxiety, and can happen at the same time as a panic attack.

When someone is hyperventilating they breathe unnaturally fast or deep. This makes the blood lose more carbon dioxide than usual, which can make them feel weak and dizzy. As they begin to breathe normally again, they should feel better.

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It is very rare for children to suffer from hyperventilation, so if you think a child is hyperventilating you should try looking for other reasons why they could be showing these symptoms.

What to look for - hyperventilation

These are the six key things to look for:

1. Unnaturally fast deep breathing
2. Attention-seeking behaviour or anxiety
3. Dizziness
4. Feeling faint
5. Trembling or tingling in the hands
6. Muscle cramps in the hands and feet

What you need to do - hyperventilation

Speak to them firmly, but be kind and reassuring.

Take them somewhere that's quiet, as this can help them control their breathing again. If you can't do this, ask any bystanders to leave or turn away.

Encourage the casualty to see their doctor about how they can learn to prevent and control hyperventilation in the future.

If you are unsure of their symptoms or if they are not improving, call 999 or 112 for medical help.

Heart

The heart pumps blood through the lungs then around the body taking oxygen to all the organs. Without oxygen the cells which make up the body will die. Find out how to use first aid to treat heart problems and what you should look out for.

Topics

Angina - Is a pain in the chest caused by a reduced blood supply to the heart muscle itself.

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Cardiac arrest – This happens when the heart stops pumping effectively leading to collapse, loss of responsiveness and the person ceasing to breathe. This is a medical emergency.

Heart attack – This happens when there is an obstruction of the blood supply to part of the heart muscle, leading to pain, or if a large part of the muscle is affected, the heart could stop beating effectively (i.e. it may lead to a cardiac arrest). Urgent medical attention is needed in an attempt to improve the blood flow to the muscle.

Shock – This happens when there is a disruption to the blood supply to part of the brain, leading to a change in function (i.e. inability to speak, use an arm or a leg. If a large part of the brain is affected the person may lose responsiveness or have a seizure). Urgent medical attention is needed in an attempt to improve blood flow to the brain.

Head and eyes

Get first aid advice for dealing with eye injuries, head injuries and headaches. Watch our informative videos to see what you need to look for.

Eye injuries - Learn about the things you need to look for if someone has a foreign object in the eye or where someone has injured or damaged their eye.

Head injuries – Find out more about the effects of head injuries including: concussion, cerebral compression, skull fracture and spinal injuries.

Headaches – Read our advice on headaches to see what to do and to get more information on when you might need to seek medical help.

Bleeding

In this section you can find out what to do about bleeding and blood loss. You can learn how to give first aid treatment for cuts, grazes, and nose bleeds as well as finding out what to do if someone is bleeding severely.

You can usually control bleeding from cuts and grazes by elevating the wound and applying pressure.

A nose bleed can be serious if someone loses a lot of blood – and severe bleeding can cause distress, lead to shock and loss of responsiveness. This advice will help to give you the confidence to know what to do when a person is bleeding severely.

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Bleeding topics:

- Cuts and grazes
- Nose bleeds
- Severe bleeding
- Shock

Cuts and grazes

Cuts and grazes are common injuries that can usually be treated at home. A cut is when the skin is fully broken, and a graze is when only the top layers of skin are scraped off.

Usually, all you need to do is clean the cut or graze to reduce infection and apply pressure and raise the injury to stop the bleeding. The wound should heal by itself in a few days.

What to look for - Cuts and grazes

If the bleeding doesn't stop, or if there's a foreign object in the cut, or you think it might be infected, then you should tell them to see a health care professional.

What to do - Cuts and grazes

- Clean the wound by rinsing it under running water or using alcohol-free wipes.
- Pat it dry using a gauze swab and cover it with sterile gauze. If you don't have these, then use a clean, non-fluffy cloth.
- Raise and support the part of the body that's injured. If it's a hand or arm, raise it above the head. If it's a lower limb, lay them down and raise the cut area above the level of the heart. This will help stop the bleeding.
- Remove the gauze covering the wound and apply a sterile dressing.
- If you think there's any risk of infection then suggest they see a health care professional.

Nose bleeds

A nose bleed is when blood flows from one or both nostrils. It's normally caused by the tiny blood vessels inside the nostrils being ruptured.

Common causes of nose bleeds include a blow to the nose, sneezing, picking or blowing the nose, and high blood pressure.

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What to look for - Nose bleeds

Most nose bleeds are minor and only last a few minutes, but they can be dangerous if someone loses a lot of blood.

If someone has had a blow to the head, the blood may appear thin and watery. This could mean that their skull is fractured and fluid is leaking from around the brain. If that happens, it is very serious and you should call 999 or 112 for emergency medical help. See advice for head injuries.

What to do - Nose bleeds

- If someone is having a nose bleed, your priority is to control the bleeding and keep their airway open.
- Get them to sit down (not lie down) as keeping the nose above the heart will reduce bleeding.
- Get them to lean forward (not backwards), to make sure the blood drains out through their nose, rather than down their throat which could block their airway.

- Ask them to breathe through their mouth and pinch the soft part of the nose, taking a brief pause every ten minutes, until the bleeding stops.
- Encourage them not to speak, swallow, cough, spit or sniff because this may break blood clots that may have started to form in the nose.
- If the bleeding is severe, or if it lasts more than 30 minutes, call 999 or 112 for medical help.

Shock

Shock (not to be confused with emotional shock) is a life-threatening condition which happens when the body isn't getting enough flow of blood.

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This means that the cells don't get enough oxygen to enable them to work properly, which can lead to damage of the vital organs like the brain and the heart.

Shock can be caused by anything that reduces the flow of blood, including:

- Heart problems, such as a heart attack, or heart failure
- Severe internal or external bleeding
- Loss of body fluids, from dehydration, diarrhoea, vomiting or burns
- severe allergic reactions and severe infection

If someone has any of the conditions above, which can reduce the circulation or blood flow, they could develop shock, so you may need to treat them for this condition as well.

What to look for - shock

If you think somebody could be suffering from shock, there are seven key things to look for:

1. Paleness of the face (pallor)
2. Cold, clammy skin
3. Fast, shallow breathing
4. Fast, weak pulse
5. Yawning or sighing
6. Confusion
7. Loss of response (in extreme cases)

What you need to do - shock

If they are showing signs of shock:

- Lay them down with their head low and legs raised and supported, to increase the flow of blood to their head. Do not raise an injured leg.
- Call 999 or 112 for medical help and say you think they are in shock, and explain what you think caused it (such as bleeding or a heart attack).
- Loosen any tight clothing around the neck, chest and waist to make sure it doesn't constrict their blood flow
- Fear and pain can make shock worse, by increasing the body's demand for oxygen, so while you wait for help to arrive, it's important to keep them comfortable, warm and calm. Do this by covering them with a coat or blanket and comforting and reassuring them
- Keep checking their breathing, pulse and level of response.

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- If they become unresponsive at any point, open their airway, check their breathing, and prepare to treat someone who has become unresponsive.

Poisoning

Poisons are substances that can cause temporary or permanent damage if too much is absorbed by the body. Get more information on the effects that different types of poisons have on the body.

Find out how to treat someone for poisoning, understand when you might need to get emergency medical help, and what to do if someone loses responsiveness.

Alcohol poisoning – Can happen when someone has drunk an excessive amount

Drug poisoning – Caused by an overdose of prescription or illegal drugs

Food poisoning – Caused by eating contaminated food

Carbon Monoxide poisoning - Caused by inhalation of this deadly gas.

Swallowed poisons – When someone ingests poisonous substances, such as: chemicals, drugs, plants, fungi or berries.

Alcohol poisoning

Alcohol poisoning is what happens to someone when they've drunk a dangerous amount of alcohol, normally in a short space of time.

Drinking too much alcohol stops the nervous system from working properly, particularly in the brain. This can severely weaken the mental and physical body functions, like sight, speech, coordination and memory.

Alcohol poisoning can also send a person into deep unresponsiveness and, at worst, can slow or even shut down their breathing, causing death.

What to look for - Alcohol poisoning

If you think someone may have alcohol poisoning, these are the key things to look for:

- a strong smell of alcohol and you may see empty bottles or cans
- confusion and slurred speech

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- vomiting
- reddened and moist face
- deep, noisy breathing
- a strong, pounding pulse
- unresponsiveness

If they are unresponsive, you also need to look for:

- shallow breathing
- weak, rapid pulse
- widened pupils that react poorly to light

What you need to do - Alcohol poisoning

- Reassure them and cover them with a coat or blanket to keep them warm.
- Check them over for any injuries, especially head injuries, or any other medical conditions.
- If they are breathing normally but are not fully responsive, place them into the **recovery position.**
- Keep checking their breathing, level of response and pulse.
- Don't make them be sick as this could block their airway and stop them from breathing.
- If you're unsure about how serious their condition is then call 999 or 112 for medical help.
- If they lose responsiveness at any point, open their airway, check their breathing and prepare to treat someone who's become unresponsive.

Drug poisoning

Poisons are substances that can cause temporary or permanent damage if too much is absorbed by the body. Poisons can be swallowed, inhaled, injected or absorbed through the skin.

Someone can get drug poisoning from taking an overdose of prescribed drugs, over the counter drugs, or illegal drugs.

But the effects will be different depending on the type of drug and how the person has taken it, such as by swallowing, inhaling or injecting.

What to look for - Drug poisoning

If you think someone may have drug poisoning, these are 10 common things to look for:

1. Stomach pain, nausea and vomiting
2. Sleepiness leading to unresponsiveness

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3. Confusion and deliriousness
4. Excitable hyperactive behaviour
5. Sweating
6. Shaking hands
7. Hallucinations – they may claim to 'hear voices' or 'see things'
8. Unusually slow or fast pulse
9. Unusually small or large pupils
10. Needle marks which may be infected

What you need to do - Drug poisoning

- Call 999 or 111 for medical help.
- If they're responsive, help them into a comfortable position and ask them what they've taken.
- Gather as much information as you can. While you wait for help to arrive, look for any packaging or containers that will help identify the drugs.
- Keep checking their breathing, pulse and level of response.
- If they lose responsiveness at any point, open their airway, check their breathing and prepare to treat someone who's become unresponsive.
- Never try to make them vomit, but if they vomit naturally then put some of this into a bag or container and give it to the ambulance – this may help them identify the drug.

Food poisoning

Food poisoning is caused by eating contaminated food. In most cases the food hasn't been cooked properly and is contaminated by bacteria such as salmonella or Escherichia coli (E. coli), which are found mainly in meat.

Someone may feel the effects of food poisoning within a few hours, and will often be sick or have diarrhoea. However, in some cases it can take up to three days.

The effects of food poisoning can make someone feel extremely ill. The most important thing is for you to keep encouraging the person to drink water so they don't get dehydrated. Most people will get better without needing treatment.

What to look for - Food poisoning

If you think someone may have food poisoning, these are the five key things to look for:

1. Feeling sick

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2. Vomiting, sometimes bloodstained
3. Stomach cramps
4. Diarrhoea
5. Headache or fever

What you need to do - Food poisoning

- If you notice any of these symptoms, tell the person to lie down and rest.
- Give them plenty of water and a bowl to use in case they are sick.
- Encourage them to drink as much water as they can, even if they can only manage regular small sips. If they have diarrhoea, it's even more important that they drink water to replace lost fluids.
- Giving them an oral rehydration solution is good to way to replace fluids lost through diarrhoea and vomiting. This solution can replace salt and other minerals which they have lost. You can buy it in a pharmacy as a sachet which you dissolve in water.
- If the person gets worse, then advise them to call their doctor or call 999 or 112 for emergency medical help.

Carbon Monoxide poisoning

If volumes of smoke or gas are inhaled or breathed in, it can be deadly. If a casualty has inhaled fumes they need immediate medical attention as they are likely to have low levels of oxygen in their blood and tissues.

Carbon monoxide is a poisonous gas. When inhaled, the gas directly prevents the red blood cells from carrying oxygen to the body's tissues and organs. It can quickly prove fatal if it is inhaled in large amounts, for example from vehicle exhaust fumes or smoke within a confined space, or even in small amounts, for example due to leakage of fumes from a faulty boiler or heater.

Carbon monoxide is difficult to detect as it has no taste or smell. Unfortunately, lots of people who have carbon monoxide poisoning may not be aware that they are affected and may wrongly blame their symptoms on other reasons. The young and the old are at increased risk. It is best to reduce the risk of poisoning by making sure that gas appliances are serviced regularly and by fitting an approved carbon monoxide detector in your home.

What to look for

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If the casualty has had exposure to low levels of carbon monoxide for prolonged periods of time, they may complain of:

- Headaches
- Confusion
- Feeling aggressive
- Nausea and vomiting
- Diarrhoea

Severe symptoms may include:

- Grey-blue skin colouration
- Rapid, difficult breathing
- Impaired level of response, leading to unresponsiveness

What you need to do

- Call the emergency services on 999 or 112 and tell them that you suspect inhalation of fumes.
- If necessary, get them away from the source of the fumes into an environment with fresh air. Do not enter the fume-filled area yourself. If the casualty is in a closed space filled with exhaust fumes, open the doors letting the fumes escape before you enter.
- Support the casualty in a comfortable position and encourage the casualty to breathe normally. Reassure them. Stay with them until the emergency services arrive.
- Treat any burns or other injuries.
- Monitor the casualty by keeping an eye on their breathing, response and pulse. If the casualty is unresponsive, open the airway, check for breathing and if necessary, perform CPR.

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Swallowed poisons

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Poisons are substances that can cause temporary or permanent damage if too much is absorbed by the body. Poisons can be swallowed, inhaled, injected or absorbed through the skin.

Swallowed poisons include chemicals, drugs, plants, fungi and berries. For more on drugs, see drug poisoning.

Dangerous chemicals include household products like bleach, which can poison or burn the body if swallowed.

Poisonous plants include foxgloves, wild arum and certain types of mushroom. Eating laburnum seeds can cause seizures.

Drugs, both prescribed or those bought over the counter, can also be harmful if someone takes too many.

What to look for - swallowed poisons

If you think someone may have swallowed poison, these are the five key things to look for:

1. Nausea and vomiting (sometimes blood-stained)
2. Cramping stomach pains
3. A burning sensation
4. Partial loss of responsiveness
5. Seizures

What you need to do - swallowed poisons

- If the person is conscious, ask them what they have swallowed, how much and when. Look for clues, like plants, berries or empty packaging and containers.
- Call 999 or 112 for medical help and tell them as much information as possible.
- Keep checking their breathing, pulse and level of response.
- If they become unresponsive, open their airway and check breathing. Follow the instructions for treating someone who is unresponsive.
- Never try to make the person vomit, but if they vomit naturally then put some of their vomit into a bag or container and give it to the ambulance. This may help them identify the poison.

Animal bites

Bites from sharp, pointed teeth cause deep puncture wounds. This can damage tissues and let germs into the skin, so you need to treat any bite that breaks the skin, to stop it getting infected.

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One infection someone might get from an animal bite is rabies, which is a serious viral infection that attacks the brain and nervous system. If an infected animal bites a human, they will pass on the virus, through their saliva.

Almost all cases of rabies happen abroad. If someone hasn't been vaccinated, then rabies is nearly always fatal, so you need to send the casualty to hospital straight away.

Tetanus (a bacterial infection) is also a potential risk after an animal bite.

What you need to do - Animal bites

- Wash the wound from the bite thoroughly with soap and warm water. This will reduce the risk of infection from an animal bite.
- Raise and support the wound and pat it dry, preferably with clean gauze from your first aid kit. Then cover it with a sterile wound dressing.
- If the wound is large or deep, then treat for bleeding and call for an ambulance.
- If you think there's a risk of rabies, then you need to get them to hospital as fast as you can.
- If the wound is dirty or they're not sure if there if they've had a tetanus jab, then tell them to see their doctor.

- If the bite is from another human, there's also a small risk of getting hepatitis or HIV/AIDS viruses. This is much more unlikely but they should still see a doctor, in case.

Blisters

Blisters are fluid-filled bumps that look like bubbles on the skin. They usually form when someone's skin repeatedly rubs against something or is exposed to heat.

When someone gets a blister, the part of the skin that is damaged leaks fluid that collects under the top layer of the skin, forming the blister.

What to look for - Blisters

Most blisters heal themselves within a few days. But seek medical help if:

- they are extremely painful
- you think they may be infected, or
- they keep coming back

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What you need to do - Blisters

- If someone has a blister, don't burst it as this can increase the risk of infection.
- Wash the skin around the blister with clean water.
- Gently pat the skin dry with a sterile gauze pad or a clean, non-fluffy material.
- If the blister was caused by something rubbing against the skin, cover it with a plaster – ideally a special blister plaster, as these have a cushioned pad that gives extra protection.

Bumps and bruises

After someone has had an injury to the skin, they'll often get a bump or a bruise. Bumps and bruises may not appear straight away but the skin can still be painful and often feel tender or swollen at first.

Bumps are swellings caused by fluid under the surface of the skin.

Bruises are bluish or purple-coloured patches that appear because tiny blood vessels beneath the skin burst and leak blood into the soft tissue. As they heal, bruises usually change to a yellowish green colour.

What you need to do

- If someone has a bump or bruise, hold something cold onto the skin straight after the injury to help it heal and reduce the pain. This could be a bag of peas or ice wrapped in a cloth, or a cold compress if you have one. Do not leave it on for more than ten minutes.
- If the injury is painful then help them to lie down and raise the area above their heart, if possible, to help slow the flow of blood to the area and so reduce the swelling.
- If a bump or bruise becomes very swollen or very painful, then you should tell them to see a healthcare professional for advice.

Burns and scalds

Burns and scalds are damage to the skin caused by heat. A burn is usually caused by dry heat, like fire, a hot iron, or the sun. A scald is caused by wet heat, like steam or a hot cup of tea.

You need to be extra careful when treating burns. The longer the burning goes on, the more severe the injury will be, and the longer it may take to heal. So you need to cool the burn as soon as possible.

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If someone has a severe burn or scald they are likely to suffer from shock, because of the fluid loss, so they will need urgent hospital treatment.

What to look for

If you think someone has a burn or scald, there are five key things to look for:

1. Red skin
2. Swelling
3. Blisters may form on the skin later on
4. The skin may peel
5. The skin may be white or scorched

What you need to do

Stop the burning getting any worse, by moving the casualty away from the source of heat.

Start cooling the burn as quickly as possible. Run it under cool water for at least ten minutes or until the pain feels better. (Don't use ice, creams or gels – they can damage tissues and increase risk of infection).

- Assess how bad the burn is. It is serious if it is:
    - larger than the size of the casualty's hand
    - on the face, hands or feet, or
    - a deep burn
- If it is serious, call 999 or 112 for emergency medical help.
- Remove any jewellery or clothing near the burn (unless it is stuck to it).
- Cover the burned area with kitchen cling film or another clean, non-fluffy material, like a clean plastic bag. This will protect from infection.
- If necessary, treat for shock (shock is a life-threatening condition, not to be confused with emotional shock).
- If you are unsure if the burn is serious then tell the person to see a doctor.

Frostbite

Frostbite happens when parts of the skin and other tissues freeze due to low temperatures. Frostbite usually affects the fingers and toes as they are the parts of the body furthest from the heart.

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If someone has severe frostbite then they might permanently lose all feeling in that part of their body. Frostbite can also lead to gangrene, when the blood vessels and soft tissues become permanently damaged leading to death of the tissue.

Frostbite usually happens in freezing or cold and windy weather. People who cannot move around are more likely to get it. Someone with frostbite will probably have hypothermia, so be prepared to treat them for that too.

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What to look for - frostbite

If you think someone has frostbite, there are four key things to look for:

1. ‘Pins and needles’ to begin with
2. Paleness, followed by numbness
3. Hardening and stiffening of the skin
4. Change in skin colour: first white, then blotchy and blue. On recovery, the skin may be red, hot, painful and blistered. If they get gangrene, the tissue may become black due to the loss of blood supply and death of the tissue.

What you need to do - frostbite

- First, encourage them to put their hands in their armpits. Then help move them indoors or to somewhere warm.
- Once inside, gently remove anything constricting like rings, gloves or boots.
- Next, warm the body part with your hands on your lap, or under their armpits. Don’t rub it though because this could damage their skin tissue. (If there is a danger of it refreezing then don’t warm it up yet as this can cause more damage).
- Place the body part in warm (not hot) water at around 40°C (104°F) and be careful not to put it near direct heat as this can cause more damage. Dry it carefully and put on a light dressing, ideally a gauze bandage from your first aid kit.
- Once you’ve done that, help them to raise their limb to reduce swelling, with cushions or a sling for instance.
- Advise them to take some painkillers if they have some (paracetamol for example).
- Then take or send them to hospital, keeping their limb raised.

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Fainting

Fainting is when someone briefly loses responsiveness, often causing the person fall to the ground. It happens because for a moment there is not enough blood flowing to the brain.

People often faint as a reaction to pain, exhaustion, hunger, or emotional stress. It is also common after someone has been standing or sitting still for a long period of time, especially if they're feeling hot.

When someone faints, their pulse slows right down but it usually picks up and goes back to normal soon afterwards.

If someone who's fainted doesn't come round after a couple of minutes, then this could be more serious.

What to look for - Fainting

There are three key signs of someone fainting:

1. Brief loss of responsiveness, often causing them to fall to the ground
2. A slow pulse
3. Pale cold skin and sweating

What you need to do - Fainting

- If someone's feeling faint, tell them to lie down.

- Kneel down next to them and raise their legs, supporting their ankles on your shoulders to help blood flow back to the brain. Watch their face for signs that they're recovering.
- Make sure that they have plenty of fresh air – ask bystanders to move away and if you're inside then ask someone to open a window.
- Reassure the casualty and help them to sit up slowly.

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- If they don’t regain responsiveness again quickly, open their airway, check their breathing and prepare to treat someone who is unresponsive.

Fever

A fever is a high temperature. When someone’s body temperature goes above the normal body temperature of 37°C (98. 6°F), this is called a fever.

Usually fevers are caused by infections or illnesses, such as a sore throat, earache, or chickenpox.

Fevers are common in children and, though worrying for parents, they often go away without treatment.

But if a young child’s temperature goes above 39°C (102. 2°F) this can be dangerous and might trigger a seizure (fit).

What to look for - Fever

These are the six key things to look for when someone has a fever:

Early signs of fever:

1. 1.High temperature - above 37°C (98. 6°F)
2. Pale skin
3. They may feel cold, with goose pimples, shivering and chattering teeth
4. Then, later:
5. Hot, flushed skin and sweating
6. Headache
7. General aches and pains

What you need to do - Fever

- If you notice some of these symptoms, take their temperature using a thermometer.
- If their temperature is above 37°C, it’s a fever.
- Help make them comfortable and keep them cool, ideally in bed with a sheet or light duvet.
- Give them plenty of cool drinks to replace any fluid they lose from sweating.
- If they’re feeling unwell, you can give them the recommended dose of paracetamol (remember - don’t give aspirin-based medication to anyone under 16).

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- Check their breathing, pulse and level of response until they're feeling better.
- If you're worried about their condition then call their local doctor's surgery, or NHS advice line for free on 111 for advice in England and Scotland (for Wales call 0845 46 47).
- If their temperature is above 39°C, call the doctor, or the NHS advice line for free on 111 for advice in England and Scotland (for Wales call 0845 46 47).
- If they seem to be getting worse then call 999/112 for emergency medical help and be prepared to treat them for a seizure.

Meningitis

Meningitis is an infection of the membranes that surround the brain and the spinal cord. Meningitis can be caused by bacteria or a virus. Anyone can get meningitis, whatever age they are, but babies and young children under five are most at risk.

Meningitis can be very serious. If you suspect meningitis you must call 999 or 112 immediately, so they can get to hospital to be treated as soon as possible.

If spotted early, and treated quickly, most people make a full recovery.

What to look for - Meningitis

If someone has meningitis, they won't usually show all the symptoms and signs at the same time. But these are the key things to look for:

- flu-like illness with a temperature
- cold hands and feet
- joint and limb pain
- blotchy or very pale skin

As the meningitis infection develops:

- severe headache
- neck stiffness
- vomiting
- eyes become sensitive to light
- drowsiness
- in babies – high-pitched moaning or a whimpering cry, floppiness, and a tense, bulging soft spot on their head (fontanelle).

Later you may see a distinctive rash of red or purple spots that doesn't fade when pressed.

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What you need to do - Meningitis

- If someone has any of the signs of meningitis, like shielding their eyes from the light, call 999 or 112 for emergency medical help straight away and treat the fever.
- Check to see if they have a rash, with most rashes if you press the spots with the side of a glass, the spots will fade – if they don't fade, call 999 or 112 for medical help at once.
- While you're waiting for help to arrive, reassure them and keep them cool.
- Keep checking their breathing, pulse and level of response.

Hypothermia

Hypothermia happens when someone's body temperature drops below 35°C (95°F). Normal body temperature is around 37°C (98. 6°F).

Hypothermia can become life-threatening quickly, so it's important to treat someone with hypothermia straight away. Severe hypothermia, when the body temperature falls below 30°C (86°F), is often fatal.

Hypothermia is usually caused by being in a cold environment for a long time. This could be from staying outdoors in cold conditions, falling into cold water, or from living in a poorly heated house. Elderly people, babies, homeless people and anyone who is thin and frail or not able to move around easily are particularly vulnerable.

What to look for - Hypothermia

These are the four key things to look for:

1. 1 .Shivering, cold, pale, and dry skin
2. Tiredness, confusion, and irrational behaviour
3. Slow and shallow breathing
4. Slow and weakening pulse

What you need to do - Hypothermia

If you notice any of these symptoms, you need to warm them up.

If they are outside, if possible get them indoors. Cover them with layers of blankets and warm the room to about 25°C (77°F). Give them something warm to drink, like soup, and high energy food, like chocolate.

- Once they have warmed up, tell them to see a doctor as soon as possible

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- If they lose responsiveness at any point, open their airway, check their breathing and prepare to treat someone who's become unresponsive.

If they are outdoors and you can't move them indoors:

1. Find something for them to lie on to protect them from the cold ground, like heather or pine branches.
2. If their clothes are wet, change them into dry clothes, if possible. Put them in a sleeping bag and cover them with blankets, if available. Make sure their head is covered too.
3. Then call 999/112 for an ambulance. If possible, don't leave them by themselves but stay with them until help arrives.
4. While you wait for help to arrive, keep checking their breathing, pulse and level of response.

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Bones and muscles

Find out how to help someone who you think may have a broken bone (also called a fracture). Watch our first aid advice video on 'How to treat broken bones' for expert advice from one of our St John Ambulance first aid trainers.

Get first aid advice to deal with an open fracture (where a bone has broken through the skin). These pages also provide first aid advice on what to look for and what to do if you suspect someone has: a dislocated joint, a spinal injury or a sprain or strain.

Topics

- Broken bones and fractures
- Dislocated joints
- Spinal injuries
- Strains and sprains

What to look for - Broken bones and fractures

The seven things to look for are:

1. Swelling
2. Difficulty moving
3. Movement in an unnatural direction
4. Alimb that looks shorter, twisted or bent
5. A grating noise or feeling
6. Loss of strength
7. Shock

What you need to do - Broken bones and fractures

- If it is an open fracture, cover the wound with a sterile dressing and secure it with a bandage. Apply pressure around the wound to control any bleeding.
- Support the injured body part to stop it from moving. This should ease any pain and prevent any further damage.

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- Once you've done this, call 999 or 112 for medical help. While waiting for help to arrive, don't move them unless they're in immediate danger.

Waiting for medical help to arrive

Protect the injured area by using bandages to secure it to an uninjured part of the body to stop it from moving. For example, fractures on the arm can be secured with a sling, and a leg with a fracture can be tied to the uninjured leg.

Keep checking the casualty for signs of shock. This does not mean emotional shock, but is a life-threatening condition, often caused by losing blood.

If they lose responsiveness at any point, open their airway, check their breathing and prepare to treat someone who's become unresponsive.

Dislocated joints

A joint is where two bones join or connect. A dislocated joint happens when bones are partly or completely pulled out of their normal position. Joints can dislocate when a strong force wrenches the bone into an unnatural position, or because the muscle violently contracts.

The most common joints that dislocate are the shoulder, knee, jaw, or joints in the thumbs or fingers.

Dislocating a joint can be really painful and cause other damage, like a break in the end of the surrounding bone or tearing of the ligaments around the joint. Sometimes it's difficult to tell whether the injury is a dislocation or a fractured bone. If you're not sure, then always treat it as broken bone.

Don't try to put the dislocated bone back into its socket as you could make it worse.

What to look for - Dislocated joints

The four signs of a dislocated joint are:

1. Strong, sickening pain
2. Not being able to move the joint
3. Swelling and bruising around the joint
4. Shortening, bending or deformity of the joint

What you need to do - Dislocated joints

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- Advise them to stay still and help them to support their dislocated joint in the most comfortable position.
- Stop the joint from moving using a bandage. For an arm injury, make a sling to support the arm. For a leg injury, use padding or broad-fold bandages.
- To give extra support for an injured arm, tie it in place by bandaging around the sling and the chest.
- Once you've stopped the joint from moving, take or send the injured person to hospital.
- Keep checking their breathing, pulse and level of response. Check the circulation beyond the bandages every ten minutes and loosen if necessary.

Spinal injury

The spinal cord connects the brain to the rest of the body. Together they make up the central nervous system.

The spine is made up of:

- the spinal cord itself
- the bones that protect the spinal cord (called vertebrae)
- the discs of tissue between the vertebrae
- the surrounding muscles
- and the nerves which branch off it, to carry messages between the brain and the rest of the body in the form of high-speed electric pulses.

The spinal cord connects to individual muscles and tells them to move (called motor function). It also connects to organs like the skin, which communicates feelings like touch, pain and heat (called sensory functions).

The greatest risk if someone has a spinal injury is that their spinal cord will be either temporarily or permanently damaged. If this happens, they'll become paralysed from the point of injury down.

The most common cause of a spinal injury is extreme force, violent twisting or bending forwards or backwards. You should be aware of the possibility of a spinal injury if someone has:

- fallen from a height, e.g. a ladder
- fallen awkwardly, e.g. while doing gymnastics
- dived into a shallow pool and hit the bottom
- fallen from a moving vehicle, e.g. a motorbike, or a horse
- been in a collapsed rugby scrum
- been in a motor vehicle which suddenly crashes

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- been hit by a heavy object falling across their back
- had an injury to the head or face.

What to look for - Spinal injury

If you think someone may have injured their spine, these are the seven key things to look for:

If the vertebrae (bones protecting the spinal cord) are damaged:

1. pain in the neck or back
2. unusual shape or twist in the normal curve of the spine
3. soreness and/or bruising in the skin over the spine

If the spinal cord is damaged:

1. loss of control over limbs – may not be able to move arms or legs
2. loss of sensation, or abnormal sensations, e.g. burning or tingling
3. loss of bladder and/or bowel control
4. breathing difficulties

What you need to do - Spinal injury

If they're responsive:

- Reassure them and tell them not to move
- Call 999/112 for an ambulance or ask someone else to call one for you
- You need to stop their head or neck from moving to prevent further damage
- To do this, kneel or lie behind their head. Rest your elbows on the ground or on your knees to keep your arms steady. Grip each side of their head, without covering the ears, to support their head in this position so that the head, neck and spine are in a straight line

- You need to support the head until emergency services can take over, no matter how long it takes for them to come. If there is someone who can help you, ask them to put rolled-up blankets, towels or clothes on either side of the head to help support it.

If they're unresponsive:

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- Open their airway using the jaw-thrust technique. To do this, put your fingertips at the angles of the jaw and gently lift to open the airway, avoiding tilting the neck
- Then check if they're breathing
- If they're breathing, continue to support their head and call 999/112 for an ambulance. If you can't maintain an open airway then carefully follow the instructions of how to put someone with a spinal injury in the recovery position.

If they're not breathing, you'll need to start CPR (cardiopulmonary resuscitation)

While waiting for help to arrive, keep checking their breathing, pulse and level of response.

Strains and sprains

Strains and sprains are common injuries which affect the soft tissues around joints – the muscles, tendons and ligaments.

They happen when the tissues are stretched, twisted or torn by violent or sudden movements, for instance if someone changes direction suddenly, or falls and lands awkwardly.

A sprain is when a ligament has been twisted or torn.

A strain is when the muscle has been overstretched and has partially torn. (A rupture is when a muscle or tendon is completely torn).

What to look for - strains and sprains

If you think someone may have strained or sprained a muscle, ligament or tendon, these are the three key things to look for:

1. Pain and tenderness
2. Difficulty moving
3. Swelling and bruising

What you need to do - strains and sprains

Remember RICE for the four steps to deal with strains and sprains:

1. **R**est
2. **I**ce
3. **C**omfortable support
4. **E**levation

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Treatment for a strain or sprain

Step 1 of 4: Rest

- Help them to sit or lie down and support in a comfortable raised position the part they've hurt.

Step 2 of 4: Ice

- To cool the area, apply a cold compress, like an ice pack or cold pad. This will help to reduce the swelling, bruising and pain. Do not leave on for more than ten minutes.

Step 3 of 4: Comfortable support

- Leave the cold compress in place or wrap a soft layer of padding, e.g. cotton wool, around the area. Tie a support bandage around it, to hold it in place, which goes up as far as the next joint on each side. For example, for an ankle injury, the bandages should go from the base of the toes to the knees.

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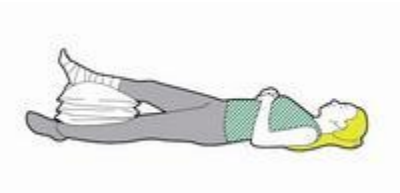
Step 4 of 4: Elevate



- Elevate the injury and support it with something soft, like cushions. If the pain is severe, or they can't use their limb at all, take or send them to hospital. Otherwise, just tell them to rest it and to see a health care professional, if necessary.

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Revision questions

Introduction, concept and principles of first aid

1. What is first aid?
2. What are the role and qualities of first aider?
3. What are features and contents of the first aid kit?
4. What do AMEGA principles stand for?
5. What do DRABC stand for?
6. how do you give first aid to someone with the following conditions:
    - Diabetes,
    - allergy,
    - fever,
    - meningitis,
    - headache,
    - ear ache and
    - tooth ache,
    - abdomen pain,
    - vomiting and diarrhoea,
    - poisons,
    - bites and stings,
    - foreign bodies,

Emergency child birth:

7. What are the Stages of labor?
8. Signs and symptoms of labor?

Management of body injuries:

9. How do you provide first aid to someone with the following conditions?

- Fractures
- dislocations
- wounds
- sprains
- strains
- head injuries
- crush injuries
- burns and scalds

Skills demonstration and practice:

10. Describe the following terms

- cardio pulmonary resuscitation (CPR)
- Artificial Ventilations,
- Recovery position,

11. List type of bandages and their function
12. How do you carry a casualty to an ambulance?

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